



path Carcinoma, lobular infiltrating
and ductal in-situ 8522/3
GSCF Carcinoma, lobular infiltrating
Site ^{DCIS} Breast NOS C50.9
path Breast, upper-outer quadrant
C50.4
9/5/13

Infiltrating Lobular Carcinoma

CATEGORY: BREAST INVASIVE CARCINOMA—INFILTRATING LOBULAR CARCINOMA

FROM PRIMARY BREAST SURGERY

CLINICAL HX - Invasive lobular carcinoma.

RIGHT BREAST, MASTECTOMY - Invasive lobular carcinoma, focally pleomorphic type, 3.0cm in greatest microscopic dimension, associated with extensive duct carcinoma in-situ (DCIS), intermediate nuclear grade, solid type with comedo necrosis. [Nottingham histologic grading system utilized- SBR score 7; architectural grade 3, nuclear grade 2, mitotic grade 2]. No lymphatic invasion is identified. Deep margin negative for tumor. Vessel wall calcifications. Intraductal papilloma, radial scar and fibrocystic changes including sclerosing adenosis. Skin and nipple, unremarkable. Prior biopsy site changes.

RIGHT AXILLARY CONTENTS, EXCISION - Metastatic carcinoma present in two of the thirteen lymph nodes (2/13). Largest metastatic focus measures 4mm in greatest microscopic dimension. No extranodal extension is identified.

Comment: AJCC pathologic stage pT2N1aMx.

Synoptic summary will follow as an addendum.

ADDENDUM 1:

RIGHT BREAST, MASTECTOMY - Immunohistochemical stain for HER-2-NEU oncoprotein is negative (performed on formalin-fixed paraffin embedded sections by manual semiquantitative morphometric analysis with appropriate positive and negative controls). No membranous staining is seen. The HER-2-NEU procedure was performed using monoclonal antibody 4B5 and DAB detection system. (Staining interpretation: Weak = 1+; Moderate = 2+; Strong = 3+.)

NOTE. This evaluation was performed according to ASCO/CAP guidelines: Positive HER2/neu cases are those with strong and complete membrane staining (3+) in greater than 30% of invasive cancer cells. Negative cases are defined as those with no staining (0) or weak, incomplete membrane staining (1+) in any proportion of cells. Equivocal cases are those cases with strong staining in less than or equal to 30% of cells, or less than strong but complete membrane staining (2+) in at least 10% of cells. Immunohistochemical stains were performed on formalin-fixed paraffin embedded tissue with appropriate positive and negative controls.

ADDENDUM 2.

Specimen Type - Mastectomy. Lymph node Sampling - Axillary dissection. Specimen Size - Greatest dimension 3cm. Histologic Type - Invasive lobular. Grading System - Nottingham. Tubule Formation - Minimal less than 10% (score = 3). Nuclear Pleomorphism - Moderate increase in size, etc (score = 2). Mitotic Count - 40X objective field with a field area of 0.152 mm²: 6 to 10 mitoses per 10 HPF (score = 2). Total Nottingham Score - Grade II: 6-7 points. Margins uninvolved by invasive carcinoma. Distance from closest margin: 5mm. Deep Primary Tumor [pT]: pT2 - Tumor greater than or equal to 2.0-5.0cm. Regional Nodes [pN]: pN1a: Metastasis in 1 to 3 axillary lymph nodes (at least 1 tumor deposit greater than 2.0mm). Number involved: 2. Number examined: 13. Distant Metastasis [pM]: pMX - cannot be assessed.

FROM DIAGNOSIS

CLINICAL HX - Right irregular mass, Right ax tail lymph node. RIGHT BREAST, 10:00 MASS, CORE BIOPSY - Invasive, lobular carcinoma with focal duct carcinoma in situ, solid type, low nuclear grade. Intraductal papilloma involved by lobular carcinoma in-situ. RIGHT AXILLA, LYMPH NODE, CORE BIOPSY - Lymph node with metastatic carcinoma histologically identical to breast carcinoma. COMMENT. Prognostic/predictive markers are being performed and results will follow as an addendum. ADDENDUM: Results of immunoperoxidase steroid receptor protein determination on paraffin sections, visually evaluated using DAB detection system and monoclonal antibodies (manual semiquantitative morphometric

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HIPAA Discrepancy		/
Print Malignancy History		/
Dual/Synchronous Primary Noted		/
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	ME	
Date Reviewed	1/28/13	

analysis): ESTROGEN RECEPTOR PROTEIN (ER- clone SP1): POSITIVE; PROGESTERONE RECEPTOR PROTEIN (PR- clone 1E2): POSITIVE. Estimated percentage of nuclei staining positive (ER): 100; Estimated percentage of nuclei staining positive (PR): 25. Intensity of staining, ER: 3+ (strong). Intensity of staining, PR: 3+ (strong). Immunohistochemical stain for HER-2-NEU oncoprotein is NEGATIVE (performed on formalin-fixed paraffin embedded sections by manual semiquantitative morphometric analysis). No membranous staining is seen. The HER-2-NEU procedure was performed using monoclonal antibody 4B5 and DAB detection system.